

ONTARIO

SUPERIOR COURT OF JUSTICE

BETWEEN:

MANZOOR UR-RAHMAN

Plaintiff

– and –

OMA DEVI MAHATOO and MOHAN
RUM MAHATOO

Defendants

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) Naresh Misir, for the Plaintiff
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) No one appearing for the Defendants
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) **HEARD:** April 17, 2014

REASONS FOR JUDGMENT

FIRESTONE J.

[1] This is an undefended trial. The defendants have been noted in default pursuant to rule 19.01(1) of the *Rules of Civil Procedure*, R.R.O. 1990, Reg.194 (“the Rules”).

[2] In this action the plaintiff Manzoor Ur-Rahman (“Rahman”) born October 16, 1949, is seeking both general and special damages following a slip and fall incident which occurred on the defendants’ driveway on January 29, 2003. The fall occurred as a result of the buildup of snow and ice.

[3] The Defendants were properly served with the trial record, notice of intention and notice of trial date as previously directed by the Court.

[4] Rahman filed for bankruptcy on April 23, 2009. He received his certificate of discharge from bankruptcy on February 5, 2010. He did on March 1, 2013, as required by rule 11.02(1) of the Rules, obtain an order to continue.

[5] The trustee in bankruptcy has confirmed that they take no issue with the trial proceeding and do not require that the title of proceedings be amended regarding the period for which the trustee claims an interest in any award made by the court.

[6] I find that the truth of all allegations of fact in the statement of claim which are deemed to be admitted pursuant to Rule 19.02(1)(a), entitle the plaintiff to judgment.

[7] The issues to be determined are the quantum of damages and whether those damages should be reduced because of any contributory negligence on the plaintiff's part.

Contributory Negligence

[8] I accept Rahman's evidence that at the time of the fall he was wearing appropriate footwear and had been walking in a careful and prudent manner. I find that the Rahman did nothing which caused or contributed to the fall. I find the plaintiff's evidence to be credible and reliable.

Damages

(a) General damages pain and suffering

[9] The evidence confirms that prior to this incident Rahman was in good physical health. He did have some pre-existing psychological issues.

[10] Following the fall Rahman was taken by ambulance to William Osler Health Centre where it was confirmed that he sustained a comminuted intertrochanteric left hip fracture.

[11] As a result on January 29, 2003 Rahman underwent emergency surgery which consisted of open reduction and internal fixation of the intertrochanteric fracture of his left hip with synthesis hip screws along with a 4-hole plate. He remained in hospital for approximately 10 days.

[12] The radiology report from Markham & Ellesmere X-Ray dated July 7, 2003 states:

"PELVIS, LEFT HIP AND FEMUR

I see a metallic processes transfixing an old communicated intertochantic fracture of the left femur. The fracture has healed. Large irregular bony mass is seen along the lesser curvature aspect. Metallic apparatus is intact and is in very acceptable position. Osteoporosis of the hip is inevitable, but no arthropathy is seen. This man should have bone density studies."

[13] Rahman was seen in consultation by Dr. Michael E. Kliman, orthopaedic on November 20, 2003. In his medical report of that date, Dr. Kliman confirms that following his discharge from hospital Rahman was required to use a walker for several months after which time he used a cane.

[14] Dr. Kliman states that the last several months he has had complaints of ongoing pain along his upper and outer thigh. He had difficulties going up stairs and walking for extended periods of time. He was unable to run. Dr. Kliman confirms he had some mild pain along his lower back.

[15] Serial x-rays up until June, 2003 showed maintenance of the fracture position and progression towards bony healing. Dr. Kliman confirms there was no evidence, on the June 2003 x-ray, of any avascular necrosis or major damage to the hip joint.

[16] Dr. Kliman states that “there is some risk that the injuries and subsequent surgeries will predispose this man early-onset arthritis along his left hip.” As well there is a moderate risk that the injuries sustained in this incident will accelerate this process on the left side.

[17] The hardware may have to be removed over the next several years if it is causing irritation or if hip replacement surgery is required. As well, according to Dr. Kliman, assuming that both hips are mildly arthritic and will eventually progress and require hip placements surgeries, in his opinion, the fracture suffered on the left side does increase the risk of future hip surgeries.

[18] Dr. Kliman states that Rahman has not recovered full strength in his left upper leg. He states “the lesser trochanteric fragment was displaced and has healed in a displaced position. This fragment contains the attachment of the hip flexor muscles and one can expect long-term weakness of hip flexion as a result. In addition, there has been a fracture through the greater trochanter which has healed in a reasonably good position, but this fragment does contain some of the hip abductor insertion and likely will result in some hip abductor weakness over the long-term.”

[19] Dr. Kliman confirms that it can be expected that there will be some long-term reduction in power, in particular, of hip flexion and to a lesser extent of hip abduction.

[20] Dr. Kliman concludes by stating “There certainly have been significant interferences with this man’s normal weight-bearing capabilities over the last year. Certainly, the first many months following the injuries, he was very limited in regards to his household and leisure activities due to his left upper leg complaints. There are still moderate limitations in regards to weight-bearing

capabilities at this time; and, although improvement should occur, there will likely be some long-term interference with certain levels of weight-bearing capabilities over the years ahead”.

[21] Rahman was seen by Dr. M.K. Joseph Kwok orthopaedic surgeon on May 22, 2009 and a report was prepared that day.

[22] In his report Dr. Kwok confirms that Rahman is still symptomatic. He confirms that his current symptoms at that time were difficulty sleeping, anxiety, depression, left hip pain left buttock thigh pain, left lower extremity weakness and stiffness and left knee pain.

[23] Dr. Kwok reports that prior to this incident, Rahman did housework. Currently he does a little housework which includes dusting, mopping, sweeping, bed making, garbage removal and laundry. Prior to this incident he did cooking and dishwashing. He does a little bit of that now.

[24] Regarding vacuuming, bathroom cleaning and kitchen cleaning he has been unable to continue with those tasks. He has resumed driving. He has not been able to resume grass cutting or snow shovelling.

[25] It is Dr. Kwok’s opinion that Rahman has not recovered completely from his hip fracture. He still experiences pain, tenderness, weakness and reduced range of motion in his left hip. As well there is evidence of mild leg length discrepancy.

[26] Dr. Kwok is further of the opinion that the present time he has limitations with respect to tasks which involve lifting, carrying, overhead reaching/lifting, repetitive reaching, pushing/pulling, frequent bending squatting, kneeling, stooping, climbing, running, and prolong sitting/standing and walking.

[27] These musculoskeletal function limitations significantly affect the plaintiff’s employment, social life and personal life. In Dr. Kwok’s opinion, it is more probable than not that these functional limitations will remain indefinitely.

[28] In his medical report Dr. Kwok states “[I] conclude that (as far as the musculoskeletal injuries sustained in a slip and fall accident are concerned) this patient is currently not disabled to return to pre-accident personal care activities. As a result of the musculoskeletal injuries sustained in the slip and fall accident this patient currently has substantial disability with respect to heavier/repetitive housekeeping tasks and home maintenance tasks. As a result of the musculoskeletal injuries sustained in the slip and fall accident this patient is not able to participate in sports activities.”

[29] Dr. Kwok goes on to state “At the present time this patient is not able to return to his pre-accident normal daily activities, housekeeping activities in-house maintenance activities etc.

without restrictions. As noted above he has a substantial disability with respect to the heavier/repetitive housekeeping tasks and home maintenance tasks at the present time.”

[30] Dr. Michael Gadon, a psychologist, saw Rahman on September 30, 2003 for the purpose of a psychological assessment. In his report dated October 20, 2003, Dr. Gadon states: “Emotionally, Mr. Ur-Rahman presented with symptoms of depression, anxiety and irritability resulting from the subject slip and fall accident and sequelae. He has become particularly depressed about his pain and physical limitations and the personal, leisure, family and occupational disruption that has occurred in his life. During his hospitalization he developed anxiety about contracting SARS but he has received medical reassurance regarding this. Psychological testing indicated that his current mood symptoms are of moderate severity and warrant a diagnosis of Adjustment Disorder. Mr. Ur-Rahman is currently being treated with mood medications and he started attending psychological counseling. These treatment modalities are helping to control his mood as well as stress related physical symptoms (e.g. stomach upset, heart palpitations, and dizziness)”.

[31] The plaintiff was seen by Dr. Howard B. Waiser, Psychologist on April 6, 2005. The Vernon-Mior and Oswestry pain questionnaires were completed. The results indicated that the plaintiff continues to experience physical distress.

[32] The plaintiff also completed Beck Anxiety Inventory and Beck Depression Inventory-II, his score was in the severe range.

[33] The plaintiff completed the Multi-Dimensional pain inventory. His response pattern indicated he was experiencing less pain than a comparable peer group. He was experiencing affective distress and according to Dr. Waiser, is looking to others to assist him in dealing with circumstances. His response profile is characterized as being dysfunctional.

[34] The plaintiff also completed the memory malingering test. He achieved a perfect score. According to Dr. Waiser based on the results achieved there was no suggestion that the plaintiff was trying to present himself as expressing cognitive distress.

[35] Dr. Waiser’s opinion is that “[h]e continues to experience significant pain which he finds to be distressing and very limiting of his ability to function as he would like. He feels that his pains are interfering with his personal life, his relationships with his family members, his work et cetera...” He “[c]ontinues to express emotional distress as a result of the accident in which he was involved. He feels limited in his working opportunities and the financial impact that this is having upon himself and his family. He was resigned to accept sedentary jobs, but noted that such positions provide him the time to reflect upon his situation and thereby aggravate his feelings of anxiety and depression. He has recently reunited with his wife, but he often

withdraws from his family because of irritability and distress. He is looking for help to assist him in dealing with his concerns, but feels that they remain significant factors in his life. Previously he has been identified as experiencing an Adjustment Disorder with Mixed Anxiety and Depressed Moods. One would concur with this diagnosis”...”[O]ne’s prognosis for him must be guarded because of his ongoing distress and the time that is passed.”

[36] Rahman was also assessed by Dr. Rakesh K. Ratti, psychologist on May 25, 2009. In addition to the clinical interview the Beck Depression Inventory-II and Beck Anxiety Inventory were administered.

[37] Dr. Ratti in his report concludes that [t]ests administered during this assessment suggests that Mr. Ur-Rahman is suffering significant affective distress, with symptoms of depressed mood and anxiety in the severe range. Given the affective distress revealed within the clinical interview and testing, it is this examiner’s clinical opinion that currently Mr. Ur-Rahman is experiencing Major Depressive Disorder, Chronic, and he exhibits significant symptoms of anxiety(the latter is not being given a formal diagnosis because it appears to have been present solely concurrent with the affective disorder).”

[38] Dr. Ratti goes on to state “[a]vailable documents indicate that the claimant was experiencing some degree of emotional distress (symptoms of anxiety and depressed mood) at times before his slip and fall, but it appears that he managed his concerns without psychological treatment. It seems that the slip and fall and its consequences(physical, emotional, and practical) have significantly exacerbated Mr Ur-Rahman’s psychological distress”...”[P]rognosis for this examinee is poor to guarded at best at this time due to the length of time he has been suffering from both physical and emotional distress and given his age.”

[39] The plaintiff testified. I found he gave his evidence in a very forthright manner. His evidence to be credible and reliable. He confirmed the physical and emotional injuries sustained which continue until today.

[40] The nature and effect of the injuries is corroborated by the evidence of the plaintiff’s wife Samina Manzoor (“Samina”) and friend Asif Wariach (“Asif”).

[41] The injuries sustained in this incident have had and will continue to have a profound effect on the plaintiff’s life.

[42] After considering the medical evidence, the plaintiff’s age and the relevant case law regarding the assessment of damages, I award the plaintiff general non-pecuniary damages in the sum of \$90,000.00.

(b) Special Damages

(1) Income Loss

[43] Rahman came to Canada in 2001.

[44] As of January 29, 2003 Rahman was working as a night security guard at Crown Security. He began there in October of 2002 and initially worked 40 hours a week and earned \$7.50 per hour.

[45] On January 9, 2003 he received a wage increase to \$8.00 per hour. Commencing January 9, 2003, he worked 20 hours per week. This enabled Rahman to accommodate a second job.

[46] In early January, 2003 the plaintiff began working as a courier during the day at Minute Man Delivery Limited as a broker on a commission basis. He worked there until January 29, 2003. His commission income for that period was \$909.45.

[47] Following the subject incident Rahman did not return to his courier position because according to his evidence, he could not keep up with the demands of the job.

[48] Rahman did on July 15, 2003 return to modified work at Crown Securities. He subsequently left Crown Securities in October 2006, because they wanted to increase his responsibilities but the work was too painful for him.

[49] Soon after he began working at Securitas Canada providing security services. Rahman had the same duties he had at Crown Securitas except he did not do patrols. He worked 40 hours per week earning between \$11.00 and \$12.00 per hour.

[50] He was let go from this position as a result of downsizing and received employment insurance benefits. He returned to Securities Canada in 2010 when work became available. In 2014 Rahman reduced his work hours to approximately 24 hours per week at the rate of \$11.82 per hour.

[51] Had the subject incident not occurred Rahman intended to continue working on a full time basis as a security guard or as a courier for as long as he was able to.

[52] The Plaintiff's past and future income losses are set forth in the economic loss report of Collins Barrow dated February 18, 2014 entered into evidence.

[53] Dr. Kwok in his medical report states: "At the present time this patient is not able to return to heavy employment tasks. As a result of the musculoskeletal injuries sustained in this accident he is not able to return to his pre-accident work as a delivery person. Because of the impairment arising from slip and fall accident his ability to continue to work in the current reduced capacity is diminished. As a result of the impairment arising from the slip and fall

accident this patient is less marketable as an employee to potential employers and this will affect his future employment opportunities and competitiveness adversely.”

[54] I am cognizant of the plaintiffs own evidence that following the incident he did not return to his position as a courier because of its physical demands.

[55] As a result I find that but for the subject incident in all likelihood he would have continued in his position as a security guard until age 67.

[56] I accept the calculations of Collins Barrow as set forth in schedule 1 of their report. I apply a 10% reduction for negative contingencies.

[57] As a result I award the sum of \$89,913.60 for past loss of income to April 17 (date of trial.)

[58] I award the sum of \$ 53,022.60 for future loss of income.

Handyman Services

[59] Rahman’s evidence is that following the subject incident he moved in with his friend Asif who assisted him with his activities of daily living. The plaintiff’s evidence is that he is limited in his housekeeping capacity.

[60] Dr. Kwok in his medical report states “[h]e has substantial disability with respect to the heavier/repetitive housekeeping tasks and home maintenance tasks at the present time. He continues to require housekeeping assistance and home maintenance assistance”.

[61] Asif’s evidence is that the plaintiff lived with him for 6-7 months. During that time he, along with his wife and children, assisted the plaintiff with his daily tasks. In return the plaintiff promised to pay him \$100.00 per week.

[62] There is no evidence from Asif that he provided these services beyond this 6-7 month period. I therefore award the sum of \$2,800.00 representing the amount the plaintiff owes to Asif for services provided.

[63] Based on the evidence presented I am satisfied that the plaintiff on balance was unable to do the heavier household and maintenance tasks for a period of time following the incident.

[64] I therefore award the sum of \$36,400.00 (\$5,200.00 a year for 7 years). This is in addition to the amount owing to Asif.

Medical Treatment

[65] Dr. Kwok in his medical report states “[t]here is evidence of ongoing musculoskeletal impairment at the present time. It is likely that he will require periodic medical and rehabilitation treatment in the future. These treatments may include physiotherapy/rehabilitation/treatments, chiropractic treatments, etc.”

[66] Dr. Ratti in his medical report states that psychotherapy is reasonably necessary given that the plaintiff’s current treatment with his psychiatrist appears to consist largely of medication monitoring.

[67] The plaintiff has medical coverage which pays \$400.00 per year. The plaintiff says he requires and would like to receive more physiotherapy, chiropractic and massage therapy than the \$400.00 will cover. Based on the medical evidence some additional treatment is warranted. The sum of \$1,500.00 is awarded for such treatment.

(2) OHIP Subrogated Interest

[68] The evidence confirms that there is an outstanding OHIP subrogated claim in the sum of \$979.90. I find this sum is payable by the defendants.

Disposition

[69] The plaintiff is to have judgment as follows:

1. General non-pecuniary damages	\$90,000.00
2. Past loss of Income	\$89,913.60
3. Future Income Loss	\$53,022.60
4. Handyman Services	\$ 2,800.00
	\$36,400.00
5. Medical treatment	\$ 1,500.00
6. OHIP Subrogated claim	<u>\$ 979.90</u>
Total:	\$274,616.10

[70] The plaintiff is entitled to pre-judgment and post-judgment interest in accordance with sections 128 and 129 of the *Courts of Justice Act*, R.R.O. 1990, c.C.43 Counsel for the plaintiff is to calculate the appropriate sum for pre-judgment interest on the various heads of damages to which prejudgment applies.

[71] In addition the Plaintiff is also granted leave to amend paragraph 1(b) of the Statement of Claim to accord with the amounts awarded herein.

[72] The Plaintiff is to provide his costs submissions totalling no more than 3 pages along with a costs outline within 15 days.

Firestone J.

Released: May 2, 2014

CITATION: Manzoor Ur-Rahman v. Oma Devi Mahatoo et al. 2014 ONSC 2636

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